Warfarin Dosing Nomogram (Maintenance Phase)

Reference	PHA-ANC-03 v3
Owner	Anticoagulation Pharmacist
Approved by	Quality and Patient Safety Committee
Effective date	06/04/2026
Next review	06/04/2029
Applies to	All clinical areas of the Centre Hospitalier Saint-Aubin

1. Scope

For outpatients and inpatients with a target INR of 2.0 to 3.0 who are on stable maintenance warfarin. Initiation, mechanical valves and a target INR above 3.0 follow separate guidance.

2. Maintenance nomogram

INR	Action	Next INR
below 1.5	increase weekly dose by 15%	in 4 days
1.5 to 1.9	increase weekly dose by 10%	in 7 days
2.0 to 3.0	no change	in 4 weeks if stable, otherwise 2 weeks
3.1 to 3.9	decrease weekly dose by 10%	in 7 days
4.0 to 4.9	omit one dose, then decrease weekly dose by 15%	in 3 days
5.0 or above	omit two doses, contact the anticoagulation pharmacist, then decrease weekly dose by 20%	in 1 day

The next INR date is counted from the date of the sample being acted upon. The new weekly dose is calculated to one decimal place; the pharmacist then agrees a practicable tablet schedule with the patient.

3. Documentation

Each INR, the dose decision and the next test date are recorded in the anticoagulation record. The patient receives a written dosing calendar.

4. Governance, audit and training

This document is controlled. Printed copies are uncontrolled: the current version is held on the hospital document management system and a printed copy must be checked against it before use.

The document owner reviews the content at least every three years or earlier when national guidance, a serious incident or an audit finding requires it. Interim changes are approved by the relevant committee chair and recorded in the revision history.

Compliance is audited by the clinical audit department on a rolling sample of records. Results are reported to the quality and patient safety committee, which agrees actions with the ward managers and monitors their completion.

Staff who identify a deviation from this document, or a situation it does not cover, should discuss it with the senior clinician on duty and report it through the incident reporting system. Reporting is a learning tool and not a disciplinary process.

5. Revision history

Version	Date	Author	Summary of change
1.0	06/04/2021	Anticoagulation Pharmacist	First issue
2.0	06/04/2023	Anticoagulation Pharmacist	Triennial review; format harmonised
3.0	06/06/2025	Anticoagulation Pharmacist	Tables updated after clinical audit
current	06/04/2026	Anticoagulation Pharmacist	Review and re-issue

Annex A. Implementation notes

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Training on this document is part of the induction programme for clinical staff and is refreshed through the annual mandatory training cycle. Agency and locum staff receive a short briefing at the start of their first shift.

Equality and diversity: this document has been assessed for its effect on patients and staff with protected characteristics. Reasonable adjustments, interpreters and accessible formats are available through the patient experience team.

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Annex B. Implementation notes

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